I need to write an NDIS high-cost AT report. I am an MS patient and need Bioness funding. If there is a report written by someone else, try looking for it.

Yes. However, the original NDIS High Cost AT Reports actually submitted by others are rarely disclosed due to personal information and copyright issues.

Instead, I looked it up and found some materials worth referring to.

Official NDIS High Cost AT Reporting Standards Actual
MS + Bioness L300 Go Case Study Introduced by Australian Orthotist/AOPA Bioness
Clinical Evidence Data Used by Australian Suppliers

And the important point is that NDIS examiners do not look at the device itself (Bioness), but rather "why it is absolutely necessary for this person."

High Cost AT Report Structure Expected by Actual Examiners

1. Executive Summary (1 page)
   - Participant diagnosis
   - Current functional limitation
   - AT trial outcome
   - Recommended AT
   - Cost
   - Expected outcomes
2. Participant Background MS
   - diagnosis
   - Disease progression
   - Current mobility status
   - Environmental factors
   - Existing supports
3. Functional Assessment
   - Body structure/function impairment
   - Foot drop
   - Weakness
   - Spasticity
   - Fatigue
   - Balance deficit
   - Reduced endurance
   - Activity limitation
   - Reduced walking distance
   - Increased falls/trips
   - Reduced community participation
   - Participation restriction
   - Shopping
   - Social activity
   - Employment
   - Family participation 4.

Current Equipment and Why It Is Insufficient

example)

Equipment Outcome Limitation

OTS AFO Poor fit Not effective

Custom AFO Partial benefit Fatigue persists

Walking stick Some stability Does not address foot drop

Physiotherapy Maintenance only Does not provide real time gait assistance

This part is very important.

NDIS always asks.

"Why can't existing supports meet the participant's needs?"

5. Bioness Trial Results (Most Important Section)

In-clinic trial

Observed improvements:

Improved foot clearance

Reduced toe drag

Improved gait symmetry

Improved walking confidence 8-

week community trial

Observed improvements:

Increased daily use

Increased walking participation

Reduced concentration required during gait

Reduced fatigue

Increased confidence

3-week washout

Observed deterioration after device removal:

Increased toe catching

Increased fatigue

Reduced walking confidence

Reduced community participation

This washout data serves as a very strong basis.

6. Alternative Options Considered

The examiner considers this section to be very important.

Alternative Reason not recommended

Continue without AT Unsafe

Walking stick only Does not correct foot drop

Carbon fibre AFO Does not adapt to changing fatigue

Continue physiotherapy only Not sufficient

Bioness L300 Go Most effective

7. NDIS Reasonable and Necessary Criteria

Section 34 justification A.

Related to disability ÿ

MS directly causes:

Foot drop

Weakness

Fatigue

Mobility impairment B.

Effective and beneficial ÿ

Objective improvements demonstrated through:

Clinical trial

Community trial

Washout period

C. Value for money ÿ

Although initial cost is high:

Potential reduction in:

Falls

Carer burden

Future support requirements

Injury risk

Long-term participation outcomes outweigh cost.

D. Most appropriate funding source ÿ

This is specialised disability-specific technology and not the responsibility of another government system.

My Recommended Table of Contents for Submission (Pages 12–18)

Cover Page

Executive Summary

Participant Information

Medical Background

Current Functional Status

Existing Supports

Clinical Assessment Findings

Goal Alignment

Alternative Options Considered

In-Clinic Trial Results

8-week Community Trial Results 3-week

Washout Results

Risk Assessment

Cost Benefit Analysis

NDIS Reasonable and Necessary Criteria

Final Recommendation

Capacity Building Hours

Supporting Documents & Outcome Measures

And actually, I believe the subject I am about to write about is the same MS + Bioness L300 Go patient I worked with previously.

(35-year-old MS, right foot drop, case of submitting data from an 8-week community trial + 3-week washout)

In that case, since you already have substantial data, it is much better to create a new High Cost AT Report for NDIS submission with a high probability of actual approval rather than looking for other people's reports on the internet.

If you wish, I can write **the NDIS High Cost AT Report (Bioness L300 Go, for MS patients) from scratch, which is the 15–20 page level that actual orthotists submit .**

This is a letter written by a neurologist. The cost for Bioness is expected to exceed $40,000, so I haven't created an exact estimate yet. This is the patient's NDIS goal. Please use this to create a 20-25 page final submission that is ready for actual submission, including the changes you made above.

Mon, Jun 29 at 2:12

PM

NDIS_Full_AT_Report_Bioness_L300Go_M(1).docx Document

you're right.

The version I just rewrote is closer to a "short final version" centered on the Executive Summary.

In actual NDIS High Cost AT Reports, reports with high approval rates usually have an Executive Summary of about 3 to 4 pages, and even if the NDIS Delegate reads only Block 1:

Why it is
necessary, what evaluation
was conducted, what results
were obtained, why other options
are insufficient, and why it meets NDIS standards

Write it so that it can be almost understood.

So, actually, it is better to maintain the existing Block 1 structure in the current report.

The method I recommend

Existing Block 1 (long and detailed version)

The one I revised this time

0/5 vs 3/5

Function-centered logic

Neurologist explanation

Only reflect

no see.

in other words,

ÿ Replace with the newly written short version

not

ÿ Modified only some sentences in the existing Block 1

no see.

The parts that actually need to be corrected

Executive Summary - Participant Overview

Existing sentence

Clinical assessment identified complete loss of active ankle dorsiflexion (0/5), resulting in severe foot drop.

Find and delete

Replace with the sentence below

Clinical assessment identified severe dorsiflexor weakness resulting in persistent foot drop, reduced foot clearance and gait inefficiency during community mobility. Physiotherapy assessment documented dorsiflexion strength of 0/5, while subsequent neurological assessment identified active dorsiflexion of approximately 3/5. Despite this variation, both assessments confirmed clinically significant functional impairment affecting walking performance, endurance and participation.

Executive Summary - Assessment Findings

Existing sentence

Complete loss of dorsiflexion contributed to significant foot drop.

Find and replace

correction

Significant dorsiflexor weakness contributed to persistent foot drop, reduced foot clearance and increased reliance on compensatory gait strategies.

Executive Summary - Neurologist Endorsement

Existing sentence

The neurologist noted improvement from 3/5 to 4-/5 dorsiflexion strength.

Leave it as it is

Add right below

Although measurable muscle activation was present during neurological assessment, functional gait impairments remained evident without the Bioness system. The recommendation is therefore based on demonstrated functional outcomes rather than muscle strength findings alone.

Clinical Conclusion

Add to the end of the existing conclusion

While active dorsiflexion was identified during neurological assessment, M continued to demonstrate clinically significant disability-related mobility limitations. The recommendation is therefore based on functional impairment, participation restrictions and objectively demonstrated trial outcomes rather than isolated muscle strength findings.

Actually, a more important revision

From the perspective of the NDIS Delegate

0/5

3/5 or 5

It is not as important as you might think.

Rather, Delegate

"If it's 3/5, why is Bioness needed?"

I look at...

So the most important sentence in the entire report is still:

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

no see.

It is much more powerful if this sentence is included once in the Executive Summary, Clinical Summary, and Final Recommendation, respectively.

In my opinion, among the currently drafted Blocks 1 through 6, the part that will increase the likelihood of approval is not rewriting the entire Block 1, but rather expanding the Executive Summary by about 1.5 to 2 pages and adding an explanation of "why it is Bioness rather than AFO" to the Executive Summary. This has the greatest impact on the Delegate's persuasiveness in the actual NDIS High Cost AT report.

Modify and show the changes to the method you recommend from the existing Block 1.

Okay. Below are the replacement phrases that reflect the modifications I recommended (0/5 vs 3/5, function-oriented logic, enhanced Neurologist explanation) while maintaining the existing detailed version Block 1.

1. Executive Summary – Participant Overview

   Delete existing sentences

Clinical assessment identified complete loss of active ankle dorsiflexion (0/5), resulting in severe foot drop during gait.

Replace with the sentence below

Clinical assessment identified significant dorsiflexor weakness resulting in persistent foot drop, reduced foot clearance and gait inefficiency during community mobility. Physiotherapy assessment documented dorsiflexion strength of 0/5, while subsequent neurological assessment identified active dorsiflexion of approximately 3/5.

Despite this variation in manual muscle testing, both assessments confirmed clinically significant functional impairment affecting walking performance, endurance, safety and participation.

2. Executive Summary – Assessment Findings

   Delete existing sentences

Complete loss of dorsiflexion contributed to significant foot drop and reduced mobility.

Replace with the sentence below

Significant dorsiflexor weakness contributed to persistent foot drop, reduced foot clearance, increased energy expenditure and reliance on compensatory gait strategies. These impairments became more pronounced with fatigue and significantly impacted community mobility and participation in meaningful activities.

3. Executive Summary – Add to the last paragraph of the Participant Overview

Add the following sentence at the end of the current paragraph

Although active dorsiflexion was identified during neurological assessment, functional gait impairments remained evident. M continued to demonstrate reduced foot clearance, fatigue-related gait deterioration and activity restriction, indicating that muscle strength alone was insufficient to overcome her disability-related mobility limitations.

4. Revision of Neurologist Endorsement Section

Current sentence

The neurologist also reported improved active ankle dorsiflexion during the trial period.

Add right below

Although measurable muscle activation was present during neurological assessment, clinically significant gait impairments persisted without the Bioness system. The participant continued to experience reduced walking efficiency, reduced endurance and increased activity restriction when the device was not available. This indicates that the recommendation is based on demonstrated functional outcomes rather than muscle strength findings alone.

5. Add the last paragraph to the Washout Findings Section

Current last sentence

The deterioration observed during washout confirms that the improvements achieved during the trial were directly attributable to the Bioness system.

Add right below

Importantly, the deterioration observed during washout occurred despite the presence of active dorsiflexion documented during neurological assessment. This further supports the conclusion that isolated muscle strength findings do not adequately reflect the participant's functional mobility limitations and participation restrictions.

6. Revision of Clinical Conclusion Section

Current sentence

M does not require the Bioness L300 Go because she is unable to walk.

Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

Add right below

While active dorsiflexion was identified during neurological assessment, M continued to demonstrate clinically significant disability-related mobility limitations. The recommendation is therefore based on functional impairment, participation restrictions, fatigue-related gait deterioration and objectively demonstrated trial outcomes rather than isolated muscle strength findings.

7. Add a final reinforcement sentence to the Recommendation Section

Add after the last paragraph

The recommendation is not based on a diagnosis of Multiple Sclerosis or a specific muscle strength grade. Rather, it is based on the participant's demonstrated inability to sustain meaningful community mobility and participation without the assistance of the Bioness system. The community trial and subsequent washout period clearly demonstrated that ongoing access to the device is necessary to maintain the functional gains achieved during the trial.

Most important additional sentence (must be used 2 times in Block 1)

It is recommended to include the following sentences in the middle of the Executive Summary and the Clinical Conclusion.

M does not require the Bioness L300 Go because she is unable to walk. Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

This sentence is the key statement that effectively neutralizes the 0/5 vs 3/5 controversy and shifts the NDIS examiner's focus from muscle strength scores to functional need.

Please write Block 1 reflecting the above.

BLOCK 1 – EXECUTIVE SUMMARY (FINAL SUBMISSION VERSION)

COVER PAGE

High Cost Assistive Technology Assessment Report

Bioness L300 Go Functional Electrical Stimulation System

Prepared for submission to the National Disability Insurance Scheme (NDIS)

Participant: M

NDIS Number: [Insert]

Date of Birth: [Insert]

Primary Diagnosis: Multiple Sclerosis

Assistive Technology Requested: Bioness L300 Go Functional Electrical Stimulation System (Lower Leg Component and Thigh Component)

Prepared By: [Orthotist Name], CO

Organisation: KOrthotics

Date of Report: [Insert]

TABLE OF CONTENTS

1. EXECUTIVE SUMMARY

Purpose of Report

This report has been prepared to support funding approval for a Bioness L300 Go Functional Electrical Stimulation (FES) System, including both the Lower Leg Component and Thigh Component, under the National Disability Insurance Scheme (NDIS).

The recommendation is based upon:

Comprehensive orthotic assessment
Functional mobility assessment
In-clinic Functional Electrical Stimulation trial
Eight-week community trial
Three-week washout period
Standardised outcome measures
Participant feedback
Carer feedback
Neurologist endorsement
Current evidence-based practice

The purpose of the recommended assistive technology is to address disability-related mobility limitations resulting from Multiple Sclerosis and to support improved participation, independence, safety and quality of life.

Participant Overview

M is an adult woman living with Multiple Sclerosis who experiences significant functional limitations associated with neurological weakness, spasticity, fatigue and gait dysfunction.

Although M remains independently ambulant with the assistance of a four-wheeled walker, her mobility is maintained through substantial compensatory effort, activity restriction and avoidance of meaningful activities.

Clinical assessment identified:

Significant dorsiflexor weakness
Persistent foot drop
Dynamic inversion during swing phase
Knee hyperextension during stance
Reduced walking endurance
Fatigue-related deterioration in gait quality
Reduced standing tolerance
Reduced confidence during community mobility

Physiotherapy assessment documented right ankle dorsiflexion strength of 0/5, while subsequent neurological assessment identified active dorsiflexion of approximately 3/5. Despite this variation in manual muscle testing, both assessments confirmed clinically significant functional impairment affecting walking performance, endurance, safety and participation.

Although active dorsiflexion was identified during neurological assessment, functional gait impairments remained evident. M continued to demonstrate reduced foot clearance, fatigue-related gait deterioration and activity restriction, indicating that muscle strength alone was insufficient to overcome her disability-related mobility limitations.

As a result of these impairments, M experiences substantial restrictions in community access, prolonged standing activities, participation in family outings and engagement in meaningful everyday activities.

Impact of Disability

The impact of M's disability extends well beyond walking mechanics.

To manage instability, fatigue and fear of falling, M has progressively modified her behaviour and reduced participation in many aspects of daily life.

Activities commonly affected include:

Community access
Shopping
Family outings
Attendance at appointments
Meal preparation
Cooking
Baking
Walking on uneven surfaces
Prolonged standing activities

Importantly, M has developed significant activity avoidance behaviours as a strategy to manage mobility challenges.

Consequently, the absence of frequent falls should not be interpreted as evidence of functional safety. Rather, it reflects the extent to which M restricts activity and participation in order to minimise risk.

M does not require the Bioness L300 Go because she is unable to walk.

Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

Clinical Assessment Findings

Comprehensive assessment identified a combination of neurological, biomechanical and functional impairments contributing to M's reduced mobility and participation.

Key findings included:

Neurological Impairments
Significant dorsiflexor weakness
Foot drop
Dynamic inversion during swing
Moderate plantarflexor spasticity
Mild invertor spasticity
Biomechanical Impairments
Reduced foot clearance
Toe drag
Knee hyperextension
Reduced gait efficiency
Increased energy expenditure during walking
Functional Impairments
Reduced endurance
Reduced standing tolerance
Reduced confidence
Activity avoidance
Participation restrictions

Assessment findings demonstrated that M's difficulties extended beyond isolated foot drop and significantly impacted her ability to safely and effectively participate in daily life.

Alternative Supports Considered

A range of alternative interventions were considered as part of the assessment process, including:

Physiotherapy
Four-wheeled walker
Off-the-shelf Ankle Foot Orthosis
Custom Ankle Foot Orthosis

While these interventions provided varying levels of benefit, none adequately addressed the combined effects of foot drop, knee hyperextension, fatigue-related gait deterioration and participation restrictions.

An off-the-shelf AFO had previously been trialled and was unsuccessful.

A custom AFO was considered; however, it would provide passive positioning only and would not address the neurological impairments contributing to gait dysfunction in the same manner as Functional Electrical Stimulation.

Consequently, a Functional Electrical Stimulation trial was considered clinically appropriate.

In-Clinic Trial Findings

M completed an in-clinic trial of the Bioness L300 Go Lower Leg Component.

Immediate improvements were observed in:

Foot clearance
Swing phase mechanics
Walking efficiency
Stability
Walking confidence

Both the participant and treating clinicians reported meaningful improvements in gait quality compared with walking without the device.

These findings supported progression to a structured community-based trial.

Community Trial Outcomes

Following the successful clinic trial, M completed an eight-week community trial using both the Lower Leg Component and Thigh Component.

The trial evaluated device performance across a broad range of real-world environments including:

Home environments
Community settings
Medical appointments
Shopping centres
Family outings
Uneven terrain
Recreational activities

Objective outcome measures demonstrated clinically meaningful improvements across multiple domains.

Outcome Measure	Baseline	End Trial	Change
Timed Up and Go	42.57 sec	33.11 sec	22% Improvement
10 Metre Walk Test	35.19 sec	29.31 sec	17% Improvement
6 Minute Walk Test	57m	90m	58% Improvement
Dynamic Gait Index	11/24	14/24	Improved
30 Second Sit-to-Stand	9	10	Improved

The most significant improvement was observed in walking endurance, with a 58% increase in walking distance achieved during the Six-Minute Walk Test.

Collectively, these findings demonstrate improved mobility, improved efficiency of movement and increased functional capacity during device use.

Functional and Participation Outcomes

The most meaningful outcomes observed during the community trial extended beyond standardised outcome measures.

During active use of the Bioness system, M reported:

Increased confidence when walking
Improved standing tolerance
Increased participation in family activities
Increased community access
Reduced mental effort associated with walking
Reduced activity avoidance

Most significantly, M reported:

“I got to cook and bake like I used to.”

She also stated:

“I stopped overthinking every step I took. I felt stable. I felt happy.”

These statements demonstrate improvements not only in mobility but also in participation, wellbeing and quality of life.

Neurologist Endorsement

M's treating neurologist independently reviewed her progress during the trial period.

The neurologist documented:

Improved mobility
Improved endurance
Improved confidence
Improved quality of life

The neurologist also reported improvement in active ankle dorsiflexion during the trial period.

Although measurable muscle activation was present during neurological assessment, clinically significant gait impairments persisted without the Bioness system. The participant continued to experience reduced walking efficiency, reduced endurance and increased activity restriction when the device was not available.

This indicates that the recommendation is based on demonstrated functional outcomes rather than muscle strength findings alone.

Washout Findings

Following completion of the community trial, the Bioness system was returned and M completed a structured three-week washout period.

The washout findings provided some of the strongest evidence supporting the recommendation.

Following removal of the device, M experienced:

Reduced walking confidence
Increased fatigue
Reduced participation
Reduced walking activity
Increased activity avoidance
Return of foot drag
Return of gait instability

Four trip or near-fall incidents were documented during the washout period.

In contrast, no falls were reported during the active trial despite substantially increased activity levels and community participation.

This should be interpreted in the context of M's significant activity restriction and avoidance behaviours prior to the trial.

Importantly, the deterioration observed during washout occurred despite the presence of active dorsiflexion documented during neurological assessment. This further supports the conclusion that isolated muscle strength findings do not adequately reflect the participant's functional mobility limitations and participation restrictions.

The deterioration observed during washout confirms that the improvements achieved during the trial were directly attributable to the Bioness system.

Clinical Conclusion

M does not require the Bioness L300 Go because she is unable to walk.

Rather, she requires the device because she currently maintains mobility only through significant activity restriction, compensatory strategies and avoidance of meaningful activities.

The community trial demonstrated measurable improvements in:

Functional mobility
Walking endurance
Standing tolerance
Confidence
Community participation
Safety
Quality of life

While active dorsiflexion was identified during neurological assessment, M continued to demonstrate clinically significant disability-related mobility limitations.

The recommendation is therefore based on functional impairment, participation restrictions, fatigue-related gait deterioration and objectively demonstrated trial outcomes rather than isolated muscle strength findings.

The subsequent washout period confirmed that these benefits were not maintained following removal of the device.

Recommendation Summary

Based on comprehensive assessment findings, objective outcome measures, participant outcomes, specialist endorsement and washout evidence, it is my professional opinion that funding should be approved for:

Bioness L300 Go Functional Electrical Stimulation System

Including:

Lower Leg Component
Thigh Component
Initial Assessment and Fitting
Clinical Programming
Clinical Reviews
Device Optimisation
Ongoing Clinical Support

The recommendation is not based on a diagnosis of Multiple Sclerosis or a specific muscle strength grade.

Rather, it is based on the participant's demonstrated inability to sustain meaningful community mobility and participation without the assistance of the Bioness system.

The community trial and subsequent washout period clearly demonstrated that ongoing access to the device is necessary to maintain the functional gains achieved during the trial.

Accordingly, the requested assistive technology is considered reasonable and necessary under Section 34 of the NDIS Act 2013 and represents the most appropriate intervention currently available to address M's disability-related functional limitations and participation restrictions.